MICHIGAN MEDICINE
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Dalbavancin Protocol for Acute Bacterial Skin and Skin-structure Infections (ABSSSI)

Current Dalbavancin Restrictions:
Dalbavancin is currently restricted to outpatient use only. First doses of dalbavancin will be allowed in the ED per the ABSSSI protocol. The ED is responsible for identifying potential candidates.

Potential Candidate Eligibility Criteria:

- Adult (≥ 18 years) in AES
- Cellulitis, wound infection, or abscess highly suspected or known to be caused by gram-positive bacteria
- Diabetic Foot Infection (non-osteomyelitis) to bridge to Podiatry Fast Track Clinic
- Patient will require admission for IV antibiotics if dalbavancin is not administered:
    - failed or unable to take PO antibiotics
    - moderate to large cellulitis in patient with comorbidities (i.e., obesity, concerns for adherence), that raise concern for successful absorption/adequacy of oral antibiotic agent planned for use

Exclusion criteria:

- Allergy to glycopeptide-related medication (vancomycin, telavancin, oritavancin, bleomycin)
- Significant immunocompromise:
    - BMT with one of the following criteria:
        - Transplant within the previous 12 months
        - Treatment for GvHD in the previous 3 months
        - Currently receiving tacrolimus
    - SOT with one of the following:
        - Transplant within the previous 12 months
        - Treatment for acute rejection in the previous 3 months
    - Neutropenia (< 500 cells/mm^3
    - Prednisone ≥ 20 mg/day (or equivalent steroid dose) in combination with other immunosuppressant agents
- Hemodynamic instability or concern for severe sepsis
- ABSSSI from or suspected to be associated with: chronic venous stasis ulcer wounds, surgical site infection, decubitus ulcer, infected burns, facial cellulitis, osteomyelitis, bacteremia, concern for necrotizing fasciitis, concern for gram negative infection or mixed infection
- Small abscess with adequate drainage performed
- Extensive bilateral erythema
- Pregnant
- Moderate to severe hepatic impairment (Child-Pugh Class B or C)
- Unable to follow up
- ID consult thought to be needed

Process:

- Patient identified as potentially eligible by clinician
- If patient if being treated for Diabetic Foot Infection (low concern for osteomyelitis)
    - Consider levofloxacin in addition to dalbavancin for gram negative coverage if:
        - Previous tissue culture with gram negative bacteria
        - Relapsed or recurrent foot infection
        - Freshwater exposure (lakes, rivers)I

Levofloxacin renal dosing		
CrCl (mL/min)	Dose	Duration
> 50	750 mg daily	7 days
30 – 49	500 mg daily	7 days
10 – 29	750 mg x1, then 250 mg daily	7 days
< 10 or HD	750 mg x1, then 250 mg daily	7 days

- Clinician opens orderset for dalbavancin (only available in the ED context under title "Dalbavancin for ABSSSI Protocol":
  - Reviews and checks off eligibility criteria and lack of exclusion criteria.
  - Dosing
    - 1500 mg for 1 dose for patients with CrCl ≥ 30 or on HD
    - 1125 mg for 1 dose for patients with CrCl < 30 and not on HD
- Upon Discharge, BPA will include:
  - Follow up: PCP (in person or virtual) vs. Podiatry Fast Track clinic (for DFI)
  - Dalbavancin note
  - Patient Education
- Observe in the ED for 30 minutes after completion of the infusion
- No stewardship approval required
- Marker should be drawn outlining the area, to help with follow up pictures. Picture should be placed in the medical record. Patient requires education that redness may extend outside of the lines in the first 24-48 hours, but pain should be decreasing and it should begin to fade (in patient instructions).

Follow up:

- 36-72 hours post-discharge from ED: PCP (in person or virtual) vs. Podiatry Fast Track clinic
- If an uncomplicated SSTI is failing to improve after a dose of dalbavancin consider:
  - Imaging to assess for underlying abscess
  - Adding gram-negative coverage with a fluoroquinolone

Dalbavancin quick facts:

- Dalbavancin provides gram-positive coverage only – if infection is thought to be due to a gram-negative organism, addition of gram-negative coverage is required
- Dalbavancin has an extremely long half-life (346 hours) - a single dose provides coverage for 7-14 days
  - Addition of alternative gram-positive agents (such as vancomycin) within 7 days of a dalbavancin dose or within 6 weeks of a 2-dose series of dalbavancin is discouraged

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References	
	Citation
10/2023	Revised immunocompromised criteria

Revision History	
Date	**Revision**
11/2024	Addition of diabetic foot pathway
04/2025	Removed ADTU

The recommendations in this guide are meant to serve as treatment guidelines for use at Michigan Medicine facilities. If you are an individual experiencing a medical emergency, call 911 immediately. These guidelines should not replace a provider's professional medical advice based on clinical judgment, or be used in lieu of an Infectious Diseases consultation when necessary. As a result of ongoing research, practice guidelines may from time to time change. The authors of these guidelines have made all attempts to ensure the accuracy based on current information, however, due to ongoing research, users of these guidelines are strongly encouraged to confirm the information contained within them through an independent source.